

RIVERSIDE ORTHOPEDIC ASSOCIATES

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OFFICE VISIT NOTE

PATIENT INFORMATION

Patient:	Maria Elena Santos	DOB:	03/14/1988
Date of Service:	February 10, 2026	Visit Type:	New Patient
Referring MD:	Dr. Jennifer Walsh (ER)	Insurance:	Hartford WC
Physician:	Kevin Park, MD		

CHIEF COMPLAINT

Right wrist fracture sustained in workplace fall on 02/08/2026.

HISTORY OF PRESENT ILLNESS

Ms. Santos is a 35-year-old right-hand dominant female carpenter who presents for evaluation of a right distal radius fracture. She fell approximately 15 feet from scaffolding at a construction site on 02/08/2026. She was seen at Riverside General Hospital ER where X-rays revealed a displaced distal radius fracture. Closed reduction was performed and the wrist was placed in a sugar-tong splint.

Patient reports persistent pain at 6/10 despite pain medication. She notes swelling has improved slightly since the ER visit. She denies numbness, tingling, or weakness in her fingers. She is unable to work and has been off work since the injury.

REVIEW OF SYSTEMS

Positive: Right wrist pain, difficulty sleeping due to pain, headaches (attributed to concurrent concussion)
Negative: No fever, no chest pain, no numbness/tingling in fingers

PHYSICAL EXAMINATION

General: Well-appearing female in no acute distress.

Right Upper Extremity:

- Splint intact, clean, and dry
- Mild swelling visible at wrist, improved from ER notes
- Fingers warm, pink, with brisk capillary refill
- Able to flex/extend all digits against resistance
- Sensation intact to light touch all digits
- Radial pulse 2+

IMAGING REVIEW

Reviewed ER X-rays from 02/08/2026:

- Pre-reduction: Displaced distal radius fracture with 25° dorsal angulation
- Post-reduction: Acceptable alignment with 5° residual dorsal angulation
- No involvement of radiocarpal joint
- Ulnar styloid intact

New X-rays obtained today (02/10/2026):

- Alignment maintained
- No interval displacement
- Early callus formation not yet visible

ASSESSMENT

1. **Displaced fracture of distal radius, right, post-reduction** - maintaining alignment
2. **Work-related injury**

PLAN

1. Convert to short arm cast today for improved comfort and stability
2. Cast to remain in place for 4-6 weeks
3. Continue current pain medications as prescribed by ER
4. Follow up in 2 weeks for repeat X-rays
5. **Work Status: TOTALLY DISABLED** - Patient cannot return to construction work until fracture healed and cleared
6. Refer to hand therapy after cast removal
7. If alignment is lost or patient develops symptoms of carpal tunnel, may need surgical intervention

WORKERS' COMPENSATION NOTES

Date of Injury:	February 8, 2026
Employer:	Titan Construction LLC
WC Claim #:	HRT-2026-0892147
Work Restrictions:	No use of right hand/arm. No lifting. No driving.
Estimated Disability:	8-12 weeks minimum
Next Evaluation:	February 24, 2026

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Board Certified Orthopedic Surgery
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FOLLOW-UP VISIT NOTE

Date of Service: March 8, 2026

INTERVAL HISTORY

Patient returns for 4-week follow-up. She reports pain has significantly decreased to 2/10. She has been compliant with activity restrictions. Headaches from concussion have resolved. She reports some stiffness in her fingers from immobilization.

PHYSICAL EXAMINATION

Cast removed for examination:

- Incision from ER laceration repair well-healed
- Wrist: Mild residual swelling, tenderness over fracture site minimal
- ROM: Limited due to 4 weeks of immobilization - expected
- Grip strength: Reduced compared to left (expected)
- Neurovascular: Intact

IMAGING

X-ray right wrist 03/08/2026:

- Fracture healing well with visible callus formation
- Alignment maintained
- No hardware (non-surgical management)

ASSESSMENT & PLAN

Distal radius fracture - healing appropriately.

1. Remove cast permanently today
2. Apply removable wrist splint - wear for activities, may remove for hygiene and gentle exercises
3. Begin occupational therapy for ROM and strengthening (prescription provided)
4. OT 2-3x per week for 6-8 weeks
5. **Work Status: MODIFIED DUTY** - May return to light duty office work only. No lifting over 5 lbs with right hand. No gripping. No construction work.
6. Follow up in 4 weeks
7. Anticipate full release to work in approximately 8-10 more weeks depending on therapy progress

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Date: March 8, 2026